

Parental Leave Policies in Obstetrics and Gynecology Training Programs in the United States and Canada

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OBJECTIVE: To assess parental benefits across accredited U.S. and Canadian obstetrics and gynecology residency programs.

METHODS: This was a cross-sectional study conducted from 2024 to 2025. All 292 residency programs in the United States were invited to participate by email, and U.S. and Canadian institutional websites were reviewed for publicly available policies. Policies were evaluated using a framework of 26 “transparency items” derived from prior work in general surgery. Descriptive statistics are reported as numbers (percentages) or medians (ranges).

RESULTS: Data were obtained from 66 of 292 U.S. programs (22.6%) and 10 of 16 Canadian obstetrics and gynecology residency programs (62.5%). Parental leave policies were obtained by direct response in 55.4% of cases and through online review in the remaining 44.6%. The median number of transparency items addressed at each program was 12 (0, 20) in U.S. programs and 14.5 (13, 18) in Canadian programs. Although most programs reported having parental leave policies, detailed information was usually lacking. The U.S. programs offered a median of 8 weeks of maternity leave (range 6–26 weeks), indicating considerable variability. Canadian programs were generally more transparent and provided longer leaves for both maternity and paternity benefits.

CONCLUSION: Parental leave policies in obstetrics and gynecology training remain inconsistent and often lack transparency.

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*A full list of the members of the Collaborative Research in Pelvic Surgery Consortium is available in Appendix 1, <http://links.lww.com/AOG/E715>.

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Many resident physicians navigate pregnancy, childbirth, and early parenthood while managing the demands of clinical training.^{1,2} This is especially relevant in obstetrics and gynecology, a specialty committed to caring for women during their reproductive years and beyond in which over 80% of residents are women. Despite Accreditation Council for Graduate Medical Education (ACGME)³ mandates and American College of Obstetricians and Gynecologists⁴ statements regarding recommended minimum durations of paid parental leave that is separate from vacation and sick time, substantial variation remains in how parental leave is operationalized across residency programs.

Prior studies, especially in general surgery, have documented inconsistent implementation, with many trainees reporting inadequate information about leave

options, perceived stigma when taking leave, and unclear or insufficient support for clinical coverage during absences.^{5–8} Transparency and accessibility of program policies surrounding parental benefits are key components of an equitable training environment. However, publicly available information on parental benefits on program websites or institutional policy handbooks may be difficult for applicants and current trainees to access and interpret.^{9,10} Studies suggest that clearer, more comprehensive policies on parental benefits can enhance trainees' perceptions of support and fairness, regardless of parental status.^{11–13}

Given the high stakes for both trainees and training programs, a systematic evaluation of the transparency and content of parental leave and benefit policies in obstetrics and gynecology residency programs is needed. Available literature describing parental leave and benefit policies in obstetrics and gynecology residency programs is largely out of date.^{14,15}

The objective of this study was to assess the transparency of parental benefit policies across accredited U.S. and Canadian obstetrics and gynecology residency programs. Specifically, we aimed to evaluate the extent to which programs provide clear and comprehensive guidance about parental benefits and whether this transparency was associated with Doximity top 20 obstetrics and gynecology program status or unionization status.

METHODS

We performed a cross-sectional study of parental leave policies from obstetrics and gynecology programs in the United States and Canada. The study was conducted by the Society of Gynecologic Surgeons CoRPS (Collaborative Research in Pelvic Surgery Consortium) research group. A list of email contacts for obstetrics and gynecology residency programs was obtained from the ACGME website Institution and Program Finder in July 2024 (www.acgme.org). An email was sent for all U.S. programs from a shared study email address to each program director and program coordinator asking for their parental leave and benefit policy. Canadian program information was obtained online given readily accessible information of provincial residency union websites. Attempts were made to contact each program with three consecutive emails over a 2-week interval, which was determined to be a reasonable response time in case respondents were out of the office. Each author was responsible for sending out the standardized email soliciting parental leave information over the designated time intervals over the study period.

When possible, personal emails to programs from study team members were also sent. If no response was received to more than three emails, the program was considered nonresponsive. Emails were sent between July and October 2024. In addition, institutional websites were searched by study team members to identify publicly posted parental leave policies between August 2024 and April 2025. The 2024 Doximity top 20 highest-ranked obstetrics and gynecology residency list by reputation was accessed on December 24, 2024,¹⁶ and program inclusion on this list was collected as a variable of interest. Doximity rankings are determined by a combination of objective data on program research productivity and boards pass rates, as well as reputation surveys of practicing physicians and program alumni. Additional data collected for each program included program location, total number of residents, method of obtaining parental leave policy, presence of program policy online, and unionization status. Unionization status was determined by program website review and review with program contacts by study team members.

Provided policies were evaluated with a framework of 26 “transparency items” derived from prior work in general surgery, including leave (11 items), paid benefits (four items), lactation accommodations (two items), contract flexibility (three items), and childcare provisions (six items) (Table 1).¹⁰ Each transparency item was identified as offered, not offered, or not reported by the provided leave policy of the program. Transparency was determined by the ability to obtain information that was available on public residency program websites. If there was information that was accessible behind firewalls, this was not considered to be transparent. All authors were involved in the data abstraction on the transparency points. The websites of all programs were perused regardless of response to the email query. A maximum of 1 point was allocated if a transparency item was addressed in the parental leave policy. If a program did not address a particular item in parental leave, then a score of 0 was allocated for that particular item. There are a total of 26 items on the originally published surgical program transparency surgical report card by Rangel et al¹⁰ in 2023, and this correlates with a maximum possible score of 26 points. Details on duration of vacation and total and paid maternity/paternity leaves were extracted.

Statistical analysis was performed with SPSS 29. Data were analyzed with descriptive and comparative statistics. Continuous data (nonparametric) were described using medians (range). Categorical data were described with number (percentage). Groups

Table 1. Twenty-six Items of Transparency

Leave
No. of total vacation days
Presence of maternity leave policy
Duration of maternity leave policy
Presence of paternity leave
Duration of paternity leave
No. of sick days
No. of professional leave days (CME, education activities, society meetings)
Reference of FMLA/government rules in policy
No. of weekday vacation days
Timing of the rest of FMLA (for second leave)
Availability of other trainees to donate sick days
Paid benefits
Duration of paid maternity leave
Duration of paid paternity leave
Presence of paid time off policy
Maintenance of benefits during leave
Lactation accommodations
Presence or mention of lactation room
Presence of lactation policy
Contract flexibility
Ability to roll vacation time to future years
Ability to roll sick days to future years
Ability to extend contract past original date
Childcare
Presence of on-site childcare
Presence of priority daycare enrollment
Presence of monetary childcare support
Presence of sick or backup childcare services
Description of childcare hours
Description of childcare availability/waitlist

FMLA, Family and Medical Leave Act.
Adapted from Rangel et al.¹⁰

(Canada vs the United States, unionized vs non-unionized, and Dximity top 20 vs others) were compared with the Mann–Whitney test, and effect size and 95% CIs were reported with the Hodges–Lehmann estimate for continuous data and χ^2 or Fisher exact test for categorical data. Significance was defined as $P<.05$.

The study was reviewed by the University of Texas at Austin IRB and deemed not human subjects research (STUDY00006457).

RESULTS

Data were obtained from 66 of 292 U.S. programs (22.6%) and 10 of 16 Canadian obstetrics and gynecology residency programs (62.5%), for an overall response rate of 24.6%, which is comparable to response rates in prior studies.¹⁰ Parental leave policies were obtained by direct response in 55.4% of cases and through online review in the remaining 44.6%. Among the programs in the United States, 21.1% were in the Northeast, 26.3% in the South,

26.3% in the Midwest, and 13.2% in the West. Of the 66 programs in the United States, 36 (54.5%) were not unionized and 17 (25.8%) were unionized, and 13 (19.7%) did not report union status. All of the Canadian programs were unionized. The median number of residents among all programs was 25 (11, 72). The median number of residents per program among unionized, not unionized, and unionization status not reported did not differ ($P=.28$). A comparison of the top 20 ranked programs in Dximity in 2024 with those not top ranked found no difference among distribution in the Northeast, South, Midwest, or West ($P=.30$). There were statistically more residents in the top-ranked Dximity programs compared with the programs that were not top ranked (36 [21, 72] vs 24 [11, 48], $P<.001$) (Table 2).

In a comparison of transparency scores of the U.S. and Canadian programs, U.S. programs had significantly lower median transparency scores among the 26 items (12 [0, 20] vs 14.5 [13,18], $P<.001$) (Table 3). However, when we further examined the subscales, the Canadian programs had increased transparency in leave duration, paid benefit, and contract flexibility but lower transparency in lactation accommodations and childcare provisions.

There was no difference among unionized and nonunionized programs in the United States regarding total transparency score or any of the subscales (Table 3). There was no difference among the top 20 ranked programs and those not among the top 20 programs regarding 26 items of transparency; however, those among the top 20 programs had significantly more transparency regarding lactation policies (0 [0, 2] vs 0 [0, 2] [$P=.03$]) and childcare provisions (0, [0, 6] vs 0 [0, 3], $P=.04$) (Table 3).

When we compared 17 unionized programs in the United States with 10 Canadian programs that are all unionized, several differences were identified (Table 4). Canadian programs had greater transparency in the subscales of leave duration, paid benefits, and contract flexibility. However, Canadian programs did not have more transparency regarding lactation and childcare compared with their unionized U.S. counterparts.

Durations of total maternity and paternity leave and paid maternity and paternity leave were significantly shorter in the U.S. unionized programs compared with Canadian programs, all of which are unionized (Table 5). The median difference in duration of maternity leave (United States vs Canada) was –54 weeks (effect size –72, –44 weeks; median 8 weeks vs 78 weeks, $P<.001$) and paternity leave by –31 week (effect size –67, –51 weeks; median

Table 2. Baseline Program Characteristics

	Unionization Status for U.S. Programs					Top 20 Doximity Status for U.S. Programs		
	Total Programs (n=76)	Unionized (n=17)	Not Unionized (n=36)	Unionization Status Not Reported (n=13)	P	Top 20 Doximity Programs (n=20)	Not a Top 20 Doximity Program (n=46)	P
Program region*								
United States	66 (86.8)							
Northeast	16 (21.1)	7 (25.9)	7 (19.4)	2 (15.4)	.002	8 (40.0)	8 (17.3)	.30
South	20 (26.3)	1 (3.7)	14 (38.9)	5 (38.5)		5 (25.0)	15 (32.6)	
Midwest	20 (26.3)	3 (11.1)	13 (36.1)	4 (30.8)		5 (25.0)	15 (32.6)	
West	10 (13.2)	6 (22.2)	2 (5.6)	2 (15.4)		2 (10.0)	8 (17.3)	
Canada	10 (13.2)	N/A	N/A	N/A	N/A	N/A	N/A	N/A
No. of residents in program†	25 (11, 72)	29 (15, 70)	29 (15, 70)	20 (12, 48)	.28	36 (21, 72)	24 (11, 48)	<.001

N/A, not applicable (all Canadian programs are unionized and not reported on Doximity rankings).

* Data are presented n (%) and compared with χ^2 or Fisher exact test if less than 5.

† Data are presented as median (minimum, maximum) and compared with the Mann–Whitney test.

6 weeks vs 62.5 weeks, $P<.001$), indicating substantially shorter leave in the United States. The U.S. programs also offered shorter duration of paid maternity leave, with a median difference of -42 weeks effect

size ($-72, -10$ weeks; median 6 weeks vs 54 weeks, $P<.001$) and paid paternity leave with a median difference of -6 weeks (effect size $-4, -34$ weeks; median 6 weeks vs 14.5 weeks, $P<.001$).

Table 3. Obstetrics and Gynecology Training Programs Adherence to 26 Points of Transparency by Region, Unionization Status, and Ranking*

Transparency Items (Median [Range])	Region		Effect Size (95% CI)	P	Unionized Excluding Canada (n=17)	
	United States (n=66)	Canada (n=10)				
Total number of transparency items (26 items)	12 (0, 20)	14.5 (13, 18)	$-4 (-6, -1)$	<.001	11 (0, 20)	
Leave duration (11 items)	8 (0, 10)	8.5 (6, 9)	$-1 (-3, 0)$	.048	6 (0, 10)	
Paid benefits (4 items)	3 (0, 4)	4 (3, 4)	$-1 (-2, 0)$	.01	3 (0, 4)	
Lactation accommodations (2 items)	0 (0, 2)	0 (0, 2)	0 (0, 0)	.28	0 (0, 6)	
Contract flexibility (3 items)	0 (0, 2)	2 (1, 3)	$-1 (-2, -1)$	<.001	0 (0, 2)	
Childcare provisions (6 items)	0 (0, 4)	0 (0, 2)	0 (0, 0)	.21	0 (0, 6)	

Transparency Items (Median [Range])	Not Unionized (n=36)	Effect Size (95% CI)	P*	Top 20 Doximity Program (n=20)	Not Top 20 Doximity Program (n=46)	Effect Size (95% CI)	P
Total number of transparency items (26 items)	12 (0, 19)	0 ($-3, 5$)	.79	12 (0, 20)	12 (0, 17)	0 ($-3, 4$)	.80
Leave duration (11 items)	8 (0, 10)	1 (0, 3)	0.16	6.5 (0, 10)	8 (0, 10)	1 (0, 2)	.07
Paid benefits (4 items)	3 (0, 4)	0 (0, 2)	0.51	1 (0, 4)	3 (0, 4)	1 (0, 2)	.64
Lactation accommodations (2 items)	0 (0, 2)	0 (0, 0)	0.09	0 (0, 2)	0 (0, 2)	0 (0, 0)	.03
Contract flexibility (3 items)	0 (0, 2)	0 (0, 0)	0.67	0.5 (0, 2)	0 (0, 2)	0 (0, 0)	.64
Childcare provisions (6 items)	0 (0, 4)	0 (0, 0)	0.33	0 (0, 6)	0 (0, 3)	0 (0, 0)	.04

Bold indicates statistical significance ($P<.05$).

* Data presented as median (minimum, maximum) and compared with the Mann–Whitney test. Effect size and 95% CIs were reported with the Hodges–Lehmann estimate for continuous data or χ^2 or Fisher exact test for categorical data.

Table 4. Unionized U.S. Versus Canadian Programs*

Transparency Items	Region		Effect Size (95% CI)	P
	United States Unionized (n=17)	Canada (n=10)		
Total number of transparency items mentioned by the policy (26 items)	6.5 (0, 20)	14.5 (13, 18)	−5 (−11, 0)	.06
Leave duration (11 items)	6 (0, 10)	8.5 (6, 9)	−2 (−5, 0)	.02
Paid benefits (4 items)	3 (0, 4)	4 (3, 4)	−1 (−3, 0)	.04
Lactation (2 items)	0 (0, 2)	0 (0, 2)	0 (0, 0)	.94
Contract flexibility (3 items)	0 (0, 2)	2 (1, 3)	−1 (−2, −1)	<.001
Childcare (5 items)	0 (0, 3)	0 (0, 2)	0 (0,0)	.94

Effect size and 95% CIs were reported with the Hodges–Lehmann estimate for continuous data or χ^2 or Fisher exact test for categorical data.
 Bold indicates statistical significance ($P<.05$).

* Data presented as median (minimum, maximum) and compared with the Mann–Whitney test.

DISCUSSION

Accessible and comprehensive parental leave policies among obstetrics and gynecology residency training programs are variable in terms of transparency. Although Canadian programs had greater transparency overall, there were no differences in transparency across the lactation and childcare subscales. The majority of programs in our study met only half or fewer of the transparency items (12–14 of 26), highlighting the overall lack of transparency in parental leave policies in obstetrics and gynecology programs across the United States and Canada.

Few studies have previously explored transparency of parental leave policies in obstetrics and gynecology residency programs, and those that have been published relied on data collected directly from residency program representatives rather than from publicly available or institutionally published data.^{14,17} Since the publication of these studies, ACGME has mandated at least 6 weeks of paid parental leave (exclusive of vacation and sick time), and the American College of Obstetricians and Gynecologists continues to reaffirm its prior recommendations. Most

recently, the ACGME Council of Review Committee Residents has made recommendations for more comprehensive parental leave and support policies.¹⁸ Our study is a more contemporary exploration of parental leave policy transparency in obstetrics and gynecology residency programs, which is important because the mandate was issued in 2022.

Our findings of low overall transparency in obstetrics and gynecology training programs are consistent with prior studies in other surgical subspecialties, which have also reported low transparency scores and limited publicly available parental leave policies.^{10,19} A prior 2023 study examining transparency items in surgical training programs identified an average of 8 of 26 transparency items,¹⁰ indicating some progress on parental leave policies, but clearly more work remains to be done.

Our study also reiterated expected differences in length of paid and unpaid parental leave policies between the United States and Canada. In the absence of a federally guaranteed paid parental leave policy, U.S. obstetrics and gynecology programs largely adhered to the ACGME mandate of 6 weeks with

Table 5. Duration of Leave and Parental Leave, U.S. Unionized Versus Canadian Programs*

Transparency Item	United States Unionized (n=17)	Canada (n=10)	Effect Size (95% CI)	P*
Duration of vacation, median (range) (wk)	4 (2, 4)	4 (2, 4)	0 (0, 0)	.69
Duration of maternity leave, median (range) (wk)	8 (6, 26)	78 (30, 78)	−54 (−72, −44)	<.001
Duration of paid maternity leave, median (range) (wk)	6 (6, 18)	54 (16, 78)	−42 (−72, −10)	<.001
Duration of paternity leave, median (range) (wk)	6 (6, 16)	62.5 (35, 78)	−31 (−67, −51)	<.001
Duration of paid paternity leave, median (range) (wk)	6 (6, 18)	14.5 (13, 18)	−6 (−4, 34)	<.001

Bold indicates statistical significance $P<.05$.

* Data presented as median (minimum, maximum) and compared with the Mann–Whitney test. Effect size and 95% CIs were reported with the Hodges–Lehmann estimate for continuous data or χ^2 or Fisher exact test for categorical data.

a median duration of paid maternity leave of 6 weeks. In contrast, Canadian programs in our study exceeded the minimum paid parental leave durations set by federal policy and had a median duration of paid maternity leave of 45 weeks and median leave duration of 61 weeks.

We initially hypothesized that unionization status would have a positive effect on parental leave and benefit policies among U.S. programs. Although unionization allows collective bargaining power and therefore allows trainees to advocate for greater benefits, the absolute numbers available for comparison were small and limited our analysis. Notably, all programs in Canada are unionized.¹⁵ This unionization status, in addition to a federally protected leave policy established by national labor protections, likely accounts for the longer leave duration across Canadian programs.

Another interesting finding in our study was the lack of difference in transparency among programs ranked in the top 20 by Doximity compared with those not ranked in the top 20. We hypothesized that the top 20 institutions would likely have more resources and possibly funding to provide better parental leave policies. Theoretically, more transparent policies would help these institutions to recruit the most talented incoming trainees. However, our data did not show any differences between overall transparency scores or length of parental leave between the top 20 programs and those outside the top 20. The only significant difference was greater transparency regarding lactation accommodations and childcare provisions in subscores among top 20 programs compared with those not in the top 20. This suggests that top 20 programs may be more specific about these services because they have more resources and therefore more availability of childcare and lactation services to offer; in addition, they may be more forthcoming about these resources because they want to attract the best trainee applicants. Although it is difficult to interpret these isolated findings, they do highlight the need for a better understanding of what drives differences in leave length and policy transparency.

Our findings should be considered against the limitations of our study. Our U.S. data from 66 of the 292 currently accredited programs represent a fraction of the full range of U.S. training programs. Although we intended to include a mix of top-ranked and non-top-ranked programs, our approach may not capture the diversity of obstetrics and gynecology training programs across the United States and their associated parental leave policies. Data extraction from publicly available websites and email communications with

programs may have also contributed to missing and inaccurate data. Website information may not have reflected the full scopes of institutional policies, and programs with better policies may have been more likely to be transparent about their practices on an online forum. In addition, personalized email outreach to programs could have influenced responses and introduced further bias.

Furthermore, our study broadly evaluated paternity and maternity leave; we did not investigate nuances of leave policies such as differences in policies for birthing and nonbirthing parents outside the traditional framework of maternity and paternity leave and policies for parents planning adoption or surrogacy. We also did not investigate program director gender or proportion of female faculty, which could add to the breadth of the analysis.

In addition, it is important to note that our study focused on transparency, not the adequacy of parental leave policies. We understand that transparent policies are not equivalent to good policies. Our findings highlight the limited transparency in parental leave policies in U.S. and Canadian training programs and, more specifically, the lack of adequate parental leave support among U.S. training programs. A study examining online publicly available parental leave policies showed only 0–11% of programs had program-specific parental leave policies available, with institutional policies more frequently available (58–83%). Unionization status was significantly associated with the presence of any parental leave policy (either program or institutional websites).¹⁹ Therefore, we need not only policies that are transparent and readily disclosed by training programs but, more precisely, policies that provide better support for trainees having children. Limited literature exists indicating that parental leave policies affect applicant decision making.²⁰ Further studies on how transparency of parental policies may affect applicant decision making are needed. According to trainees' perspectives in a systematic review, ideal parental leave policies offer a minimum of 6–8 weeks of leave, with a formal and clearly written policy available.¹³ It is important to keep these variables in mind in planning future studies to improve access and adequacy of parental leave policies for obstetrics and gynecology trainees.

Our study demonstrates the overall low transparency in parental leave policies across obstetrics and gynecology training programs in the United States and Canada. Our work adds to the current limited body of literature on parental leave policies and benefits in obstetrics and gynecology training programs.

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PEER REVIEW HISTORY

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